ADULT CORONARY THROMBOSIS

GUIDELINES 2025
EUROPEAN RESUSCITATION COUNCIL©

1. Prevent & be prepared



Promote cardiovascular health to reduce the risk of acute coronary events



Enhance health education to recognise symptoms & BLS training for likely rescuers



Strengthen regional STEMI networks to ensure timely PCI

2. Suspect & react



Evaluate 12-lead ECG after ROSC; repeat if inconclusive



Assess clinical data suggesting acute coronary syndrome



Activate the STEMI network & transfer the patient to PCI centre is ST-elevation or ongoing ischemia

3. Resuscitate & establish reperfusion strategy

Patients with sustained ROSC

ST-elevation

Is PCI possible within 120 min?

Yes
Immediate coronary angiography ± PCI

No
Consider fibrinolysis if not contraindicated & transfer to PCI centre

No ST-elevation

Is the patient unstable or shows signs of ongoing ischemia?

Yes
Immediate coronary angiography ± PCI

No
Consider ruling-out non-coronary causes

Consider delayed coronary angiography ± PCI

No Sustained ROSC

Does the OHCA setting suggest likely futility?

Yes
Consider stopping CPR

No
Consider transfer to PCI centre with ongoing CPR

Consider mechanical CPR

Consider ECPR

BLS – basic life support; CPR - cardiopulmonary resuscitation; ECPR - extracorporeal cardiopulmonary resuscitation; STEMI – ST elevation myocardial infarct; PCI – percutaneous coronary intervention; ECG – electrocardiography; OHCA – out-of-hospital cardiac arrest; ROSC – return of spontaneous circulation